

LF-126284-01



Pack Insert For Malaysia

NeoMercazole®

Carbimazole

Composition

Active ingredient: Carbimazole

Each NeoMercazole 5mg tablet contains carbimazole
Ph. Eur. 5mg.

Excipients:

Lactose, Maize Starch, Gelatin, Magnesium Stearate,
Sucrose, Acacia, Talc, Red Iron Oxide (E172).

Pharmaceutical form

NeoMercazole 5mg tablets are pink, circular biconvex,
imprinted with Neo 5 on the obverse and plain on the reverse.

Properties and effects

Pharmacodynamic properties

ATC Code: H03B B

Pharmacotherapeutic group: Sulfur-containing
imidazole derivatives

Mechanism of action:

Carbimazole, a thionamide, is a pro-drug which
undergoes rapid and virtually complete metabolism to
the active metabolite, thiamazole, also known as
methimazole. The method of action is believed to be
inhibition of the organification of iodide and the coupling
of iodothyronine residues which in turn suppress the
synthesis of thyroid hormones.

Pharmacokinetic properties

Absorption

Carbimazole is rapidly metabolised to thiamazole.
After oral ingestion, peak plasma concentrations of
thiamazole, the active moiety, occur at 1 to 2 hours

Distribution

The total volume of distribution of thiamazole is 0.5
l/kg. Thiamazole is concentrated in the thyroid gland.
This intrathyroidal concentration of thiamazole has the
effect of prolonging its activity. However, thiamazole
has a shorter half-life in hyperthyroid patients than in
normal controls and so more frequent initial doses are
required while the hyperthyroidism is active.

Biotransformation

Thiamazole is moderately bound to plasma proteins.
Carbimazole has a half-life of 5.3 to 5.4 hours. It is
possible that the plasma half-life may also be prolonged
by renal or hepatic disease. See section Posology and
method of administration. Thiamazole crosses the
placenta and appears in breast milk. The plasma milk
ratio approaches unity.

Elimination

Over 90% of orally administered carbimazole is
excreted in the urine as thiamazole or its metabolites.
The remainder appears in faeces. There is 10%
enterohepatic circulation.

Therapeutic indications

NeoMercazole is an anti-thyroid agent. It is indicated in
adults and children in all conditions where reduction of
thyroid function is required.

1. Hyperthyroidism.
2. Preparation for thyroidectomy in hyperthyroidism.
3. Therapy prior to and post radio-iodine treatment.

Posology and method of administration

For oral administration only.

NeoMercazole should only be administered if
hyperthyroidism has been confirmed by laboratory tests.

Posology

Adults

The initial dose is in the range 20mg to 60mg taken as
two to three divided doses. The dose should be titrated
against thyroid function until the patient is euthyroid in
order to reduce the risk of over-treatment and resultant
hypothyroidism. Subsequent therapy may then be
administered in one of two ways.

Maintenance regimen: Final dosage is usually in the
range 5mg to 15mg per day, which may be taken as a
single daily dose. Therapy should be continued for at
least six months, and up to eighteen months. Serial
thyroid function monitoring is recommended, together
with appropriate dosage modification in order to maintain
a euthyroid state.

Blocking-replacement regimen: Dosage is maintained at
the initial level, i.e. 20mg to 60mg per day, and
supplemental L-thyroxine, 50mcg to 150mcg per day, is
administered concomitantly, in order to prevent
hypothyroidism. Therapy should be continued for at least
six months, and up to eighteen months. Where a single



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Proof Number: 1
Revision Date: 25-Apr-2023
Revised by: DAJ

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Commodity No.: 28067419
Pharma Code: 28067419
Print Colours: Black
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dosage of less than 20 mg is recommended, it is intended that carbimazole 5 mg tablets should be taken.

Elderly

No special dosage regimen is required, but care should be taken to observe the contraindications and warnings as it has been reported that the risk of a fatal outcome to neutrophil dyscrasia may be greater in the elderly (aged 65 or over).

Children

Use in children and adolescents (3 to 17 years of age)
The usual initial daily dose is 15mg per day adjusted according to response.

Contra-indications

Hypersensitivity to the active substance or to any of the excipients

Serious, pre-existing haematological conditions, severe hepatic insufficiency.

Patients with a history of acute pancreatitis after administration of carbimazole or its active metabolite thiamazole.

Special warnings and special precautions for use

Bone marrow depression including neutropenia, eosinophilia, leucopenia and agranulocytosis has been reported. Fatalities with carbimazole-induced agranulocytosis have been reported.

Rare cases of pancytopenia/aplastic anaemia and isolated thrombocytopenia have also been reported. Additionally, very rare cases of haemolytic anaemia have been reported.

Patients should always be warned about the onset of sore throats, bruising or bleeding, mouth ulcers, fever and malaise and should be instructed to stop the drug and to seek medical advice immediately. In such patients, white blood cell counts should be performed immediately, particularly where there is any clinical evidence of infection.

Following the onset of any signs and symptoms of hepatic disorder (pain in the upper abdomen, anorexia, general pruritus) in patients, the drug should be stopped and liver function tests performed immediately. Early withdrawal of the drug will increase the chance of complete recovery.

Women of childbearing potential and pregnancy:

Women of childbearing potential have to use effective contraceptive measures during treatment. The use of

carbimazole in pregnant women must be based on the individual benefit/risk assessment. If carbimazole is used during pregnancy, the lowest effective dose without additional administration of thyroid hormones should be administered. Close maternal, foetal and neonatal monitoring is warranted (See Pregnancy and lactation).

There have been post-marketing reports of acute pancreatitis in patients receiving carbimazole or its active metabolite thiamazole. In case of acute pancreatitis, carbimazole should be discontinued immediately. Carbimazole must not be given to patients with a history of acute pancreatitis after administration of carbimazole or its active metabolite thiamazole. Re-exposure may result in recurrence of acute pancreatitis, with decreased time to onset. NeoMercazole should be used with caution in patients with mild-moderate hepatic insufficiency. If abnormal liver function is discovered, the treatment should be stopped. The half-life may be prolonged due to the liver disorder.

NeoMercazole should be stopped temporarily at the time of administration of radioiodine (to avoid thyroid crisis).

Patients unable to comply with the instructions for use or who cannot be monitored regularly should not be treated with NeoMercazole.

Regular full blood count checks should be carried out in patients who may be confused or have a poor memory. Precaution should be taken in patients with intrathoracic goitre, which may worsen during initial treatment with NeoMercazole. Tracheal obstruction may occur due to intrathoracic goitre.

The use of carbimazole in non- pregnant women of childbearing potential should be based on individual risk/benefit assessment (see Pregnancy and lactation). There is a risk of cross-allergy between carbimazole, the active metabolite thiamazole (methimazole) and propylthiouracil.

Carbimazole may cause white cell disorders such as neutropenia and agranulocytosis, which may be fatal if treatment with carbimazole is not stopped promptly. These reactions usually occur during the first 3 months of therapy, and in most cases, are reversible on stopping treatment.

Since agranulocytosis can develop very rapidly, periodic leucocyte counts alone may not be effective in the early detection of these reactions.



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NeoMercazole contains lactose. Patients with rare hereditary problems of galactose intolerance, the total lactase deficiency or glucose- galactose malabsorption should not take this medicine.

Interaction with other medicaments and other forms of interaction:

Little is known about interactions.

Interaction studies have not been performed in paediatric patients.

Particular care is required in case of concurrent administration of medication capable of inducing agranulocytosis.

Since carbimazole is a vitamin K antagonist, the effect of anticoagulants could be intensified. Additional monitoring of PT/INR should be considered, especially before surgical procedures.

The serum levels of theophylline can increase and toxicity may develop if hyperthyroid patients are treated with antithyroid medications without reducing the theophylline dosage.

Co-administration of prednisolone and carbimazole may result in increased clearance of prednisolone.

Carbimazole may inhibit the metabolism of erythromycin, leading to reduced clearance of erythromycin.

Serum digitalis levels may be increased when hyperthyroid patients on a stable digitalis glycoside regimen become euthyroid; a reduced dosage of digitalis glycosides may be needed.

Hyperthyroidism may cause an increased clearance of beta-adrenergic blockers with a high extraction ratio.

A dose reduction of beta blockers may be needed when a hyperthyroid patient becomes euthyroid.

Incompatibilities:

None known.

Fertility, Pregnancy and lactation

Pregnancy:

Carbimazole crosses the placenta but, provided the mother's dose is within the standard range, and her thyroid status is monitored, there is no evidence of neonatal thyroid abnormalities.

Studies have shown that the incidence of congenital malformations is greater in the children of mothers whose hyperthyroidism has remained untreated than in those who have been treated with carbimazole.

However, cases of congenital malformations have been observed following the use of carbimazole or its active metabolite methimazole during pregnancy.

A causal relationship of these malformations, especially choanal atresia and aplasia cutis congenita (congenital scalp defects), to transplacental exposure to carbimazole and methimazole cannot be excluded. Therefore, the use of carbimazole in non-pregnant women of childbearing potential should be based on individual risk/benefit assessment (see Special warnings and special precautions for use). Cases of renal, skull, cardiovascular congenital defects, exomphalos, gastrointestinal malformation, umbilical malformation and duodenal atresia have also been reported. Therefore, carbimazole should be used in pregnancy only when propylthiouracil is not suitable. If NeoMercazole is used in pregnancy the dose of NeoMercazole must be regulated by the patient's clinical condition. The lowest dose possible should be used, and this can often be discontinued three to four weeks before term, in order to reduce the risk of neonatal complications. The blocking-replacement regimen should not be used during pregnancy since very little thyroxine crosses the placenta in the last trimester.

Hyperthyroidism in pregnant women should be adequately treated to prevent serious maternal and foetal complications.

Carbimazole is able to cross the human placenta. Based on human experience from epidemiological studies and spontaneous reporting, carbimazole is suspected to cause congenital malformations when administered during pregnancy, particularly in the first trimester of pregnancy and at high doses. Reported malformations include aplasia cutis congenita, craniofacial malformations (choanal atresia; facial dysmorphism), exomphalos, oesophageal atresia, omphalo-mesenteric duct anomaly, and ventricular septal defect. Carbimazole must only be administered during pregnancy after a strict individual benefit/risk assessment and only at the lowest effective dose without additional administration of thyroid hormones. If carbimazole is used during pregnancy, close maternal, foetal and neonatal monitoring is recommended (see section Special warnings and special precautions for use).

Breastfeeding

NeoMercazole is secreted in breast milk and, if treatment is continued during lactation, the patient



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should not continue to breast-feed her baby.

Women of childbearing potential:

Women of childbearing potential have to use effective contraceptive measures during treatment (see section Special warnings and special precautions for use).

Effects on ability to drive and use machines:

The effect on the ability to drive and use machines is not known.

Undesirable effects

Adverse reactions usually occur in the first eight weeks of treatment. The most frequently occurring reactions are nausea, headache, arthralgia, mild gastric distress, skin rashes and pruritus. These reactions are usually self-limiting and may not require withdrawal of the drug.

Paediatric population

Frequency, type and severity of adverse reactions in children appear to be comparable with those in adults.

Blood and lymphatic system disorders

Bone marrow depression including neutropenia, eosinophilia, leukopenia and agranulocytosis has been reported. Fatalities with carbimazole-induced agranulocytosis have been reported.

Rare cases of pancytopenia/aplastic anaemia and isolated thrombocytopenia have also been reported. Additionally, very rare cases of haemolytic anaemia have been reported.

Patients should always be warned about the onset of sore throats, bruising or bleeding, mouth ulcers, fever, malaise and should be instructed to stop the drug and to seek medical advice immediately. In such patients, blood cell counts should be performed immediately, particularly where there is any clinical evidence of infection.

Generalised lymphadenopathy.

Immune system disorders

Angioedema and multi-system hypersensitivity reactions such as cutaneous vasculitis, liver, lung and renal effects occurs.

Endocrine disorders

Insulin autoimmune syndrome (with pronounced decline in blood glucose level)

Nervous system disorders

Headache, neuritis, polyneuropathy.

Vascular Disorders

Bleeding

Gastrointestinal system disorders

Acute pancreatitis, nausea, mild gastrointestinal

disturbance. Loss of sense of taste has been observed.

Acute salivary gland swelling.

Hepato-biliary system disorders

Hepatic disorders, including abnormal liver function tests, hepatitis, cholestatic hepatitis, cholestatic jaundice and most commonly jaundice, have been reported: In these cases carbimazole should be withdrawn.

Skin and subcutaneous tissue disorders

Skin rashes, pruritus, urticaria. Hair loss has been occasionally reported.

Severe cutaneous hypersensitivity reactions have been reported in both adult and paediatric patients, including Stevens-Johnson syndrome (very rare including isolated reports: severe forms, including generalised dermatitis, have only been described in isolated cases).

Musculoskeletal and connective tissue disorders

Isolated cases of myopathy have been reported. Patients experiencing myalgia after the intake of carbimazole should have their creatine phosphokinase levels monitored.

General disorders and administration site conditions

Fever, Malaise

Injury, poisoning and procedural complications

Bruising

Overdose and treatment

No symptoms are likely from a single large dose, and so no specific treatment is indicated.

Shelf life

2 years.

Special precautions for storage

Do not store above 30°C. Store in the original container.

Nature and contents of container

NeoMercazole tablets are available in HDPE bottles with a tamperproof ring and a round poly-propylene tamper-evident screw cap with integrated desiccant. Each bottle contains 100 tablets.

Special precautions for disposal

No special requirements

Medicine: Keep out of reach of children

Current at: Apr 2023

Manufactured by

Cenexi, 52 rue Marcel et Jacques Gaucher, 94120 Fontenay Sous Bois, France

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